



Policy

Acute Response Mental Health Emergencies / Crisis

Policy code	EMHS: 17
Original endorsement date	July 2016
Date of current endorsement	April 2024
Version	2.0
Functional subgroup	Mental Health
Summary	Mental health services should endeavour to provide all levels of acute response based on consumer needs.
Policy owner	Executive Director Safety Quality and Consumer Engagement
Contact	
Review date	April 2027
NSQHS Standards (second edition)	Standard 1 clinical governance; Standard 2 partnering with consumers
NSMHS Standards	11.2.9, 11.2.12
Impact Statement Declaration (ISD)	The consultation through the Aboriginal community is reflected in this policy to ensure cultural appropriateness and positive health impacts on the Aboriginal community (ISD Record ID)
Status	Active

1. Background

1.1. About this document

Mental health services within the EMHS catchment area should endeavour to provide all levels of active response based on individual consumer needs at the time and in accordance with the [Crisis Triage Rating Scale](#) (CTRS) completed on the Statewide Standardised Clinical Documentation (SSCD) Triage form on WebPSOLIS. This scale is to be used for all age groups.

Business hours:

The service is conducted by local community mental health Assessment Treatment Teams (ATT), Clinical Treatment Teams (CTT) and Older Adult Teams (OA) as well as other state wide services facilitating care to consumers e.g. Mental Health Emergency Response Line ([MHERL](#)), [Wungen Kartup](#)

After hours:

- Acute presentations can present for triage at the nearest emergency department.
- Staff aware of a consumer whom they consider to be at risk of a mental health emergency after business hours should alert the relevant ATT.
- If consumer is at imminent risk of harm to self, harm to others or harm from others, staff will contact the Ambulance or Police Services as appropriate.
- Acute presentations will be triaged at the nearest Emergency Department.
- ATT respond to consumer contacts up to 22:00 hrs.
 - After 22:00 hrs, acute response / crisis calls are triaged by the [MHERL](#).

1.2. Key definitions

Mental health services utilise the [Triage to Discharge Mental Health Framework for SSCD Crisis Triage Rating Scale](#) to determine the urgency of response (UoR) required for a consumer and to clearly indicate which service is required to act on the UoR.

Category	Definition
A Immediate	Immediate response requiring police/ambulance or other service e.g., overdose, siege, imminent violence.
B Within 2 hours	See within 2 hours/present to psychiatric emergency service or emergency department e.g., acute suicidality, threatening violence, acute severe non-recurrent stress.
C Within 12 hours	See within 12 hours e.g., distressed, suicidal ideation of moderate to severe nature, disturbed behaviour.
D Within 48 hours	See within 48 hours e.g., client / carer distress, early symptoms of relapse, person known to the service requiring priority review.
E Within 2 weeks	Non urgent. A routine referral is for a non-urgent assessment which will be

	managed at the next intake meeting.
F	Requires further triage contact/follow up. Community clinicians to provide a formal or informal referral as required to an alternative service provider to attend to a particular type of service.
G	No further action required. Community clinician to provide consultation, advice and/ or brief counselling if required.

2. Acute Response Mental Health Emergencies / Crisis

All people when experiencing a mental health crisis or deemed to be in a mental health emergency situation will be provided with the appropriate contact needs and assessment based on the information made available at the time. Staff will endeavour to collate as much information as is required to determine an appropriate response and record as a Triage Contact within PSOLIS. The Triage Contact will include a Risk Assessment Management Plan (RAMP). The information gathered can be from multiple sources such as the consumer / family / friends / other resources (where consent is gained) including previous records as well as PSOLIS and paper medical records.

All MH service actions / responses will be in accordance with the CTRS, taking into account the services' ability at that time to respond. Assistance should be gained from other services i.e., mental health, police and ambulance as deemed appropriate.

Services must ensure they ask Aboriginal consumers if they would like significant members of their community, including elders and traditional healers, involved in their mental health assessment, examination and treatment and their preferred process for involvement.

2.1 Category A- Immediate

2.1.1 Children and adolescents

Children and adolescents assessed as **Category A** will be referred using the identified procedure to Ambulance and / or Police Service.

- Urgent specialist advice can be provided by contacting the [Child and Adolescent Health Service | CAHS - CAMHS Crisis Connect](#).
- Children and adolescents under sixteen years of age are to be referred to their local Emergency Department via Ambulance or Police Services.
- Adolescents between the ages of sixteen and eighteen years are to be referred to the Armadale Hospital Emergency Department, Royal Perth Hospital Emergency Department, Fiona Stanley Hospital Emergency Department, Peel Health Campus Emergency Department or Rockingham General Hospital Emergency Department via the Ambulance or Police services.

- Children and adolescents who require protective arrangements and/or have been the subject of physical, sexual and/or emotional abuse are to be referred directly to the Department of Children's and Family Services, Perth Children's Hospital (PCH) Child Protection Unit and/or the Police Child Protection Squad.

Refer to [Mandatory Reporting Web System \(MRWeb\)](#) and [Guidelines for Protecting Children 2020](#) for reporting of suspected child sexual abuse.

2.1.2 Adults and older adults

Adults and older adults assessed as **Category A** are to be referred to Ambulance or Police services in accordance with the [MP0063/17 Road Based Transport for Mental Health Consumers Policy](#).

2.2 Category B, C, D - Within 2 hrs, 12 hrs and 48 hrs

2.2.1 Children and adolescents

Children and adolescents assessed as UoR **Category B; C, & D** will be referred:

- Where the degree of risk of harm to self or others is high, the receiving clinician is to take immediate action to involve the Child and Adolescent Mental Health Service (CAMHS). CAMHS is to be informed about the urgent referral and a plan for follow-up documented. CAMHS will assist with the coordination of other government and non-government mental health services.

2.2.2 Adults and older adults

Adults and older adults assessed as **Category B, C or D** will be referred using the identified procedure.

- Adults and older adults who require an acute response are to be referred to the relevant Assessment and Treatment Team (ATT), or Older Adult service of the consumer's geographical catchment area.
- During office hours, ATT will discuss acute referrals with the referring person in order to coordinate the most appropriate action with the assistance of members of the Community Treatment Team.
- The ATT and Older Adult services will process all new referrals and re-referrals of past clients. This involves:
 - Receiving the initial information
 - Completing a Triage form including the assessment of UoR required
 - Checking if the person has had previous contact with the service
 - Obtaining collateral information, and
 - Documenting all pertinent information to be taken to the next Intake Meeting.
- If ATT are unable to respond within the recommended timeframe the consumer will be referred to the nearest Emergency Department.

3. Documentation

Refer to [Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation](#)

Refer to [MP0099/18 Updated June 2021 Community Health Status Assessments Role of MH Clinician](#)

Refer to [PSOLIS State-wide Standardised Clinical Documentation \(SSCD\) Triage Business Operating Rules](#)

4. Compliance monitoring and Legislative Penalties

4.1. Compliance Monitoring

Each EMHS Site/Service Executive Director is to ensure compliance with this policy.

Compliance monitoring will include but is not limited to the following:

- Number of referrals per Triage category.
- Number of referrals seen within each Triage code timeframe.

4.2. Legislative Obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

5. Legislation

- Mental Health Act (WA) 2014
- WA Aboriginal Health and Wellbeing Framework 2015-2030
- Work Health and Safety Act 2020
- Guardianship and Administration Act 1990 (WA)

6. Additional references

6.1. WA Health Policies

- [Mental Health Triage Tool](#)
- [MP0181/24 Safety Planning for Mental Health Consumers](#)
- [MP0099/18 Updated June 2021 Community Health Status Assessments Role of MH Clinician](#)

6.2. EMHS

- [Care Coordination in Mental Health Services Policy](#)
- [East Metropolitan Health Service MH Emergency Response Line](#)

6.3. Others

- [Child and Adolescent Health Service | CAHS - CAMHS Crisis Connect](#)
- [Policy for Reporting of Notifiable Incidents to the Chief Psychiatrist – Public Mental Health Services 2018](#)
- Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation